

The HEARD Study: Healthcare Equity, Access, and Research in Diabetes – A National Survey of Community Priorities

Steven L Trawley¹, Amanda Stephens², Clare Wilson¹, Grant Brinkworth³

1 The Cairnmillar Institute, Melbourne, VIC
2 Monash University, Faculty of Medicine, Nursing and Health Sciences, Melbourne, Victoria, Australia
3 Diabetes Australia, New South Wales, Glebe, New South Wales, Australia

Asked what researchers should work on, Australians living with diabetes put affording care first – and what they name as the top priority depends on how secure their finances are.

1 Background and aim

- Diabetes research agendas are usually set by researchers and health professionals, not by the people living with the condition.
- Community priorities are usually reported by diabetes type alone, overlooking how financial circumstances shape them.

Aim

To identify the research priorities of Australian adults living with diabetes, and to examine how those priorities vary by diabetes pathophysiology and financial wellbeing.

2 Method

- National cross-sectional online survey of 981 Australian adults (aged 18+) with self-reported diabetes, run from 13 August to 28 September 2025, recruited via a research panel (70%) and paid social media advertising (30%).

Priorities were elicited with a single open-text question:

“Besides finding a cure, what do you think researchers should focus on to help people with diabetes live well every day?”

- Inductive thematic coding by a locally hosted large language model, audited blind by a second model (78.2% agreement), then reviewed by three authors and adjudicated.

All inference ran offline on local hardware; no participant text left the device.

- 125 responses (12.7%) contained no codeable research priority and were excluded, leaving 856 for analysis.
- Financial wellbeing: five-item CFPB Financial Well-Being Scale (0–100), split at the midpoint of 50.

Who took part

Mean age 54.4 (SD 15.0) years, 66% women, 72% type 2 and 25% type 1 diabetes, 39% using continuous glucose monitoring.

In their words

“I’d LOVE a pump, it would solve all my diabetes issues but being in financial hardship means I won’t ever be able to have one”

Type 1 diabetes, financially vulnerable

“Long lasting insulin pumps without having to count carbs”

Type 1 diabetes, financially secure

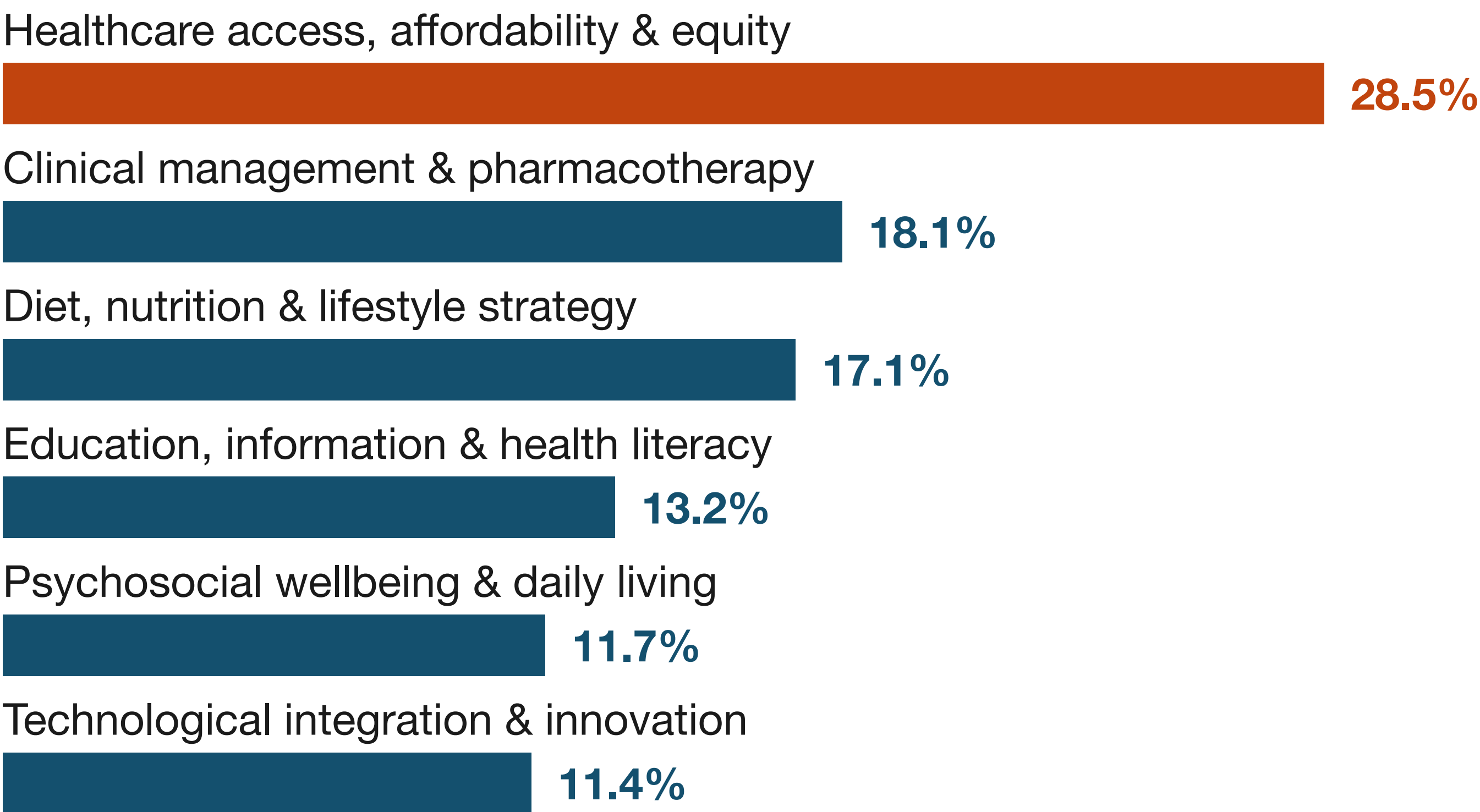
3 Results

28.5%

named healthcare access, affordability and equity their top research priority — the leading theme of six.

n = 244 of 856 substantive responses; 95% CI 25.6–31.6.

The six research priorities, in ranked order (N = 856)



Each response was assigned to one primary theme, so percentages sum to about 100%.

Within that leading theme, 65.2% of responses (159 of 244) concerned the cost and subsidisation of diabetes technology.

Financial wellbeing changed what came first

% naming healthcare access their top priority, by diabetes group and financial wellbeing



For financially secure participants with insulin-deficient diabetes, technology led instead (28.2%).

$\chi^2 = 112$, $df = 15$, $p < 0.001$. The gradient held across tertiles of financial wellbeing (34.4% to 23.1%, $p = 0.002$) and after weighting to NDSS population benchmarks (27.1%).

4 Conclusion

- Healthcare access, affordability and equity was the dominant research priority — but financial wellbeing, not diabetes type alone, shaped which priorities emerged.
- Research agendas and funding should address structural barriers to care alongside biomedical advances, informed by the financial circumstances of people living with diabetes.

